



THE CHARTERED
INSURANCE INSTITUTE

ADVANCED DIPLOMA IN INSURANCE

556 – LIFE AND DISABILITY CLAIMS

EXAMINATION GUIDE

APRIL 2009

SPECIAL NOTICE

Candidates entered for the October 2009 examination should study this Examination Guide carefully in order to prepare themselves for the examination.

Practice in answering the questions is highly desirable and should be considered a critical part of a properly planned programme of examination preparation.

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IMPORTANT GUIDANCE FOR CANDIDATES

Introduction

The purpose of this Examination Guide is to help you to understand how examiners seek to assess the knowledge and skill of candidates. You can then demonstrate to the examiners that you meet the required levels of knowledge and skill to merit a pass in this unit. During your preparation for the examination it should be your aim not only to ensure that you are technically able to answer the questions but also that you can do justice to your abilities under examination conditions.

Before the examination

Make sure you have a copy of the current Certificate/Diploma/Advanced Diploma in Insurance Information for Candidates

Details of administrative arrangements and the regulations which form the basis of your examination entry are to be found in the current Certificate/Diploma/Advanced Diploma in Insurance Information for Candidates brochure, which is essential reading for all candidates; it is available online at www.cii.co.uk or from Customer Service.

Study the syllabus carefully

It is important to study the syllabus, which is available online at www.cii.co.uk or from Customer Service. The questions in the examination paper are based directly on the syllabus, so it is vital that you are familiar with it.

Read widely

Your knowledge should be wider than the scope of one book. While books specifically produced to support your studies will provide coverage of the syllabus areas, you should be prepared to read around the subject. A reading list can be found at the end of the syllabus.

Make full use of the Examination Guide

The best way to understand what the examiners require is to study the CII Examination Guides, and you can obtain copies online at www.cii.co.uk. You can use Examination Guides as 'mock' examination papers, attempting them under examination conditions as far as possible, and then comparing your answers to the model ones. The examiners' comments on candidates' actual performance should be noted.

Understand the nature of assessment

Each Examination Guide contains a full examination paper and model answers. The model answers show the type of responses the examiners are looking for, and which would achieve maximum marks. However, you should note that there are alternative answers to some question parts which would also gain high marks. For the sake of clarity and brevity not all of these alternative answers are shown.

Know the structure of the examination

Familiarise yourself with the structure of the examination paper and the time allowed to complete it. This information can be found on the question paper included within each Examination Guide.

In the examination

Do justice to yourself in the examination

Assuming you have prepared adequately, you will only do justice to yourself in the examination if you follow two common sense rules:

1 Spend your time in accordance with the allocation of marks as indicated on the paper. If you do not complete the whole paper, your chances of passing may be reduced considerably. Do not spend excessive time on any one question. If you have used up the time allocation for that question, leave some space, go on to the next question, and only return to the incomplete question after you have completed the rest of the paper. The maximum marks allocated to each question and any constituent parts are given on the paper; the number of marks allocated is the best indication of how much time you should spend answering it. If part of a question has just two marks allocated, there are likely to be only one or two points for which the examiner is looking, so a long answer is a waste of time. Conversely, if part of a question has, for example, eight marks allocated, a couple of lines will not be an adequate answer.

2 Take care to answer the precise question set. You will see that the model answers provided in this Examination Guide are quite focused and precise; alternative answers would only be acceptable if they still answer the question. However brilliantly you write on a particular topic, if it does not provide a satisfactory answer to the precise question as set, you will not score the marks allocated. Many candidates leave the examination room confident that they have written a 'good' paper, only to be mystified when they receive a disappointing result. Often, the explanation for this lies in a failure to think carefully about what the examiner requires, before putting pen to paper.

Order of tackling questions

Tackle the questions in whatever order you feel most comfortable with. Generally, it is better to leave any questions which you feel less confident in answering until you have attempted those with which you are more familiar, but remember not to spend excessive time on your 'good' questions.

Handwriting

Provided your handwriting is legible, you will not lose marks if it is untidy. We recommend that you do not write in block capitals, because you will be slowed down so much by doing so and, paradoxically, block capitals can become more difficult to read than joined-up writing when done quickly.

Answer format

Unless the question requires you to produce an answer in a particular format, such as a letter or a report, you should use 'bullet points' or short paragraphs, since this allows you to communicate your thoughts in the most effective way in the shortest time. The model answers give an indication of which style is acceptable for the different types of question.

Calculators

If you bring a calculator into the examination room, it must be a silent, battery or solar powered, non-programmable calculator. The use of electronic equipment capable of being programmed to hold alphabetical or numerical data and/or formulae is prohibited. You may use a financial or scientific calculator, provided it meets these requirements. It is important that you show all the steps of any calculation in your answer. The examination is testing your ability to carry out all the appropriate steps in calculating a value. A proficient mathematician is someone who follows the correct method, i.e. carries out the appropriate steps. The majority of the available marks will be allocated for demonstrating the correct method of calculation.

After the examination

All Advanced Diploma in Insurance examiners, one of whom will mark your answer book, are either active practitioners in the insurance industry or are experts on the subject. They have been specially trained to mark papers using a detailed marking scheme: the model answers in Examination Guides are based on those marking schemes.

The marking of each examiner is closely monitored throughout the marking period and all marked answer books are carefully checked. This process means that all answer books are marked to the same standard.

After all the answer books have been marked, a moderation meeting is held, at which all available statistical information is considered, together with the views of the Senior Examiner for that unit and other assessment experts. At the meeting a pass mark is set to ensure that the standard of knowledge and skills required to gain a pass in the paper is comparable with that of previous papers. All candidates at or above the agreed pass mark will pass: the CII does not operate a quota system whereby only a fixed percentage of candidates can pass a paper.



THE CHARTERED INSURANCE INSTITUTE

ADVANCED DIPLOMA

APRIL 2009 EXAMINATION PAPER

**UNIT 556
LIFE AND DISABILITY CLAIMS**

INSTRUCTIONS

- Three hours are allowed for this paper.
- Fill in the information requested on the answer booklet and on form B.
- You are allowed to write on the inside pages of this question paper but you must **NOT** write your name, candidate number, PIN or any other identification anywhere on this question paper.
- The answer booklet and this question paper must be handed in personally by you to the invigilator before you leave the examination. Failure to do this may result in your paper not being marked and you may be prevented from entering this examination in future.

READ THE INSTRUCTIONS OVERLEAF CAREFULLY BEFORE ANSWERING ANY QUESTIONS.

THE CHARTERED INSURANCE INSTITUTE

556 – Life and disability claims

CANDIDATE INSTRUCTIONS

READ THE INSTRUCTIONS BELOW BEFORE ANSWERING ANY QUESTIONS.

Three hours are allowed for this paper. You should answer all questions in Part I, the compulsory question in Part II and three out of the five questions in Part III.

The paper carries a total of 200 marks, as follows:

Part I	8 compulsory questions	48 marks
Part II	1 compulsory question	50 marks
Part III	3 questions selected from 5	102 marks

You are advised to spend no more than 45 minutes on Part I.

Answer each question on a new page. If a question has more than one part, leave several lines blank after each part.

It is important to show each step in any calculation, even if you have used a calculator.

You may find it helpful in some places to make rough notes in the answer booklet. If you do this, you should cross through these notes before you hand in the booklet.

PART I

**Answer ALL questions in Part I.
Each question is worth six marks.**

Note form is acceptable where this conveys all the necessary information.

1. (a) Explain briefly the product control cycle. (3 marks)
(b) Outline what contribution the claims function should make to the product control cycle. (3 marks)
2. Explain briefly the term 'days of grace' in the context of a life assurance policy. (6 marks)
3. (a) Define epilepsy. (2 marks)
(b) Describe briefly the two major ways in which epilepsy might be classified. (4 marks)
4. (a) Define the terms 'domicile' and 'residence'. (3 marks)
(b) Outline the effect of s.19 of the Revenue Act 1889. (3 marks)
5. Outline briefly the Binet staging system and its relevance to the Association of British Insurers' standard critical illness definitions. (6 marks)
6. Outline the guidance that the Association of British Insurers' Statement of Best Practice for Critical Illness Cover provides regarding the use of generic policy exclusions. (6 marks)

7. (a) Explain what troponin is. (3 marks)
- (b) Outline how troponin is referenced in the latest Association of British Insurers' definition of heart attack. (3 marks)
8. Describe how pension payments should be treated when applying the limitation of benefit condition on an income protection claim. (6 marks)

Part II

FOR USE WITH QUESTION 9 ONLY

Home visit report dated 1 April 2009

- During the interview the claimant showed obvious signs of Parkinson's – which clearly limited his ability to undertake clerical tasks and impaired his mobility.
- He explained that he had done no work at all during the period 1 March to 31 March 2008 – however after that period his treatment became more effective resulting in a reduction of his symptoms, and an existing client persuaded him to restart work.
- Since 1 April 2008:
 - He has employed a full-time trainee – and he oversees this trainee's work on a daily basis;
 - He has been able to work 6 hours a day – compared to his pre-disability level of 12 hours a day;
 - His wife has become actively involved with the business. She undertakes all client visits and marketing, and has been instrumental in increasing the work undertaken. She works approximately the same hours as Mr Jones, and the financial split of the company is now equal between him and his wife;
 - His entitlement to State benefits ceased when he returned to work.
- Mr Jones is a genuinely motivated and honest individual who provided information openly. He has shown great resourcefulness in keeping his business going despite his illness, and trying to maintain a good quality of life for him and his family. He was genuinely unaware that he should have advised the insurance company of his return to work.
- He is aware that his condition will deteriorate and at that time he hopes to decrease his involvement with the business – and that the existing trainee, his wife, and potentially other employees will keep the business going as his input decreases.

Part II

FOR USE WITH QUESTION 9 ONLY

Profit and loss statement		
Mr & Mrs Jones – IT Design Partnership		
Accounts	Year ended 1 March 2008	Year ended 1 April 2009
Turnover	£67,000	£84,000
Wages – Trainee	£0	£15,000
Cost of sales	£2,000	£2,000
Electricity	£500	£500
Car expenses	£3,000	£3,000
Advertising	£1,000	£3,000
Phone	£500	£500
Net profit	£60,000	£60,000

Note – Accountant confirms that there was no business activity and no earnings or expenses in the period 1 March 2008 to 31 March 2008.

PART II

Compulsory question. This question is worth 50 marks.

9. Mr Jones is a 50-year-old computer software designer currently claiming under an income protection insurance policy with your office. Basic details are set out below.

Date of policy commencement	1 January 2000
Benefit level	£24,000 per annum
Deferred period	4 weeks
Expiry age	60
Definition of disability	Own occupation
Limitation of benefit clause	Maximum benefit payable will be: 50% of pre-disability earnings in the 12 months prior to disability, less State benefits received, other insurances and any continuing income.
State benefits received	£5,000 per annum
Date of disability	1 March 2008
Insured occupation	Computer software designer

Mr Jones' claim was admitted at the full level of benefit with effect from 1 April 2008. Claims evidence at the date of admittance established the following.

- There was no non-disclosure.
- A diagnosis of Parkinson's disease had been confirmed.
- Mr Jones was unable to perform his own occupation due to symptoms associated with Parkinson's disease and moderate depression associated with the diagnosis.
- Prior to disability the claimant had been working 12 hours a day as a self-employed computer software designer, with 50% of his time travelling to clients and attending meetings, and the other 50% working from home on his computer.
- Although Mrs Jones was included in the business accounts this was purely for tax reasons and prior to her husband's disability she had taken no part whatsoever in the running of the business.
- There were no other insurances.
- There was no continuing income since the claimant had been incapacitated.

Due to an administration error by the insurance company the claim was not reviewed at all until now. A home visit report and a profit and loss statement have just been received and you are now responsible for the on-going management of the claim.

The home visit report and the latest profit and loss statement are provided on pages 6 and 7. These documents should be read before answering the questions below.

- (a) Define Parkinson's disease and describe the typical symptoms, treatment, and basis for the diagnosis. (13 marks)
- (b) Explain how the maximum level of benefit payable should have been calculated when the claim was originally assessed at the outset of the claim in April 2008. (13 marks)
- (c) On receipt of the home visit report and April 2009 accounts, describe how the financial assessment of this claim would now change. (14 marks)
- (d) Outline a claims management action plan for the future of this claim. (10 marks)

PART III

**Answer THREE of the following FIVE questions.
Each question is worth 34 marks.**

10. You receive a letter from Mr Smith who has two insurance policies with your company. One is a life with critical illness policy, the other is a life with terminal illness policy. He advises you that he has just been diagnosed with cancer and wants to know what benefits he can claim and how to proceed. Both policies commenced in December 2007.

Prepare a written response to the policyholder, outlining the key points that need to be covered. (34 marks)

11. (a) State what elements typically make up the remuneration package of a company director of a publicly quoted company. (6 marks)
- (b) Explain how an assessor would establish the pre-disability earnings of such a person in the event of a claim under an individual income protection contract. (28 marks)

12. The Law Commission has been reviewing insurance law.

Outline what they have done and discuss the main areas that have been highlighted as requiring reform. (34 marks)

13. An income protection (IP) policyholder currently working as an accountant in London notifies his insurer of his impending move to Romania where he is to become an aid worker for two years. He informs his insurer that he will be working with children who have been orphaned as a result of HIV, and will receive a salary of £500 per month whilst he is there.

Outline what his insurer should advise him regarding the implications for his IP policy. (34 marks)

14. Whilst considering a death claim, evidence has come to light suggesting material non-disclosure. The claimant's representative states that a full and accurate disclosure was made to the sales agent at the time the application form was completed.

(a) Discuss the options open to the insurer if the agent was:

- a tied agent;
- an independent financial adviser. (28 marks)

(b) Describe two other agency relationships that may arise at claims stage and the implications that they may have on disclosures made. (6 marks)

NOTE ON MODEL ANSWERS

The model answers given are those which would achieve maximum marks. However, there are alternative answers to some question parts which could also gain high marks. For the sake of clarity and brevity not all of these alternative answers are shown. An oblique (/) indicates an equally acceptable alternative answer.

EXAMINER'S COMMENTS

There was a variable standard of entries for this session. Candidates who were familiar with all aspects of the syllabus were able to attain high scores, but it was not uncommon for candidates to perform poorly in relation to questions that tested knowledge of less common illnesses or more obscure areas of claims management.

Many candidates failed to deliver the level of detail required to attain good scores in Part III questions. These questions carry a significant proportion of the marks for this paper, but all too often candidates gave only brief answers that failed to address the subject matter of the question in its entirety. Candidates are reminded to pay heed to the marks awarded for each question and ensure that they devote their efforts and answers accordingly.

Question 1

This was a straightforward question affording good marks to most candidates.

Question 2

Despite some confusion with 'cooling off' periods most candidates were able to explain that an insurer remained on risk during the 'days of grace' following non payment of a premium. Very few made the point that the policy would be able to continue without further underwriting if the outstanding premiums were paid within this period.

Question 3

Almost all candidates were familiar with the nature and types of epilepsy, and hence most candidates scored well on this question.

Question 4

Other than a handful of candidates who got their answers the wrong way round, this question was generally well answered by candidates.

Question 5

There was a significant range of answers in response to this question – although many knew that the Binet system related to the staging of cancer, most were unable to state which specific cancer, and some were confused with scales of coma depth, degrees of blindness etc.

Question 6

This question was poorly answered – perhaps reflecting candidates lack of knowledge of important documents that appear as appendices within the coursebook text. Most relied on common sense principles in attempt to answer this question but this was generally not sufficient to address the specific nature of this question.

Question 7

Most candidates were familiar with troponin although most thought it was an enzyme rather than a protein – however marks were awarded for recognising it as a chemical marker reflecting heart muscle damage.

Full marks for the second part of the question relied on candidates knowing the exact levels at which the ABI defines troponin levels in the definition of Heart Attack.

Question 8

This question was poorly answered by most candidates – some were unable to give any comment on how pensions are treated in the limitation of benefit and most failed to distinguish between pensions that arise as a result of the claimant's incapacity, and those that were being received prior to this time.

Question 9

- (a) Most candidates were familiar with the diagnosis, nature and treatment of Parkinsons disease and scored well on this question.
- (b) Although most candidates were able to apply a basic limitation of benefit calculation in respect of this scenario, very few explored the position of the claimant's wife in the pre incapacity period. Examiners expected candidates to comment on how to apportion the profit of the business between husband and wife – and what principles and considerations influenced this decision.
- (c) The financial position following the claimant's return to work confused many candidates. Many were unable to determine what level of income the claimant should be considered to be in receipt of = and there was some confusion amongst candidates who thought that this was 'continuing income' that needed to be deducted from the limitation of benefit – rather than included in a proportionate benefit calculation. Few candidates mentioned that it would be prudent to make any future payments 'on account' as the claimant's situation is likely to change as time progresses, and accurate financial evidence will only be available in retrospect. Consideration of any overpayment was commonly overlooked.
- (d) Most candidates were able to make sensible suggestions relating to the future management of this claim – reflecting the unpredictable financial situation of the claimant – and the fairly predictable medical decline.

Question 10

This was a relatively straightforward question and afforded candidates good marks from basic claims management communication skills. For the maximum level of marks to be awarded examiners expected candidates to give the full cancer and terminal illness definitions (noting the relevant date of policy commencement) – along with a detailed overview of the relevant claims processes. An ideal response would include advising the claimant of the need for relevant consent to obtain appropriate medical reports, the steps in the claims assessment process, and the basic parameters of the cover under each policy.

Question 11

This was not a popular question – but those who did attempt it reflected a good basic knowledge of salary and dividend remuneration. Few mentioned the importance of how the policy conditions actually defined earnings for such individuals – and fewer still debated the scenario of dividend payments in the event of the employing company being in a loss making position. Similarly, analysis of other remunerative items such as pension contributions, benefits in kind etc was limited.

Question 12

A very unpopular question – only 1 candidate attempted it.

Question 13

This was a popular question and enabled candidates to demonstrate their knowledge of agency implications – however where candidates provided debate as to the categorisation of non disclosure and the appropriate remedies marks were awarded where relevant.

Model answer for question 1

- (a) The product control cycle relates to how a product is devised and then amended once experience is gained from a range of different areas. In devising a new insurance product, it is usual practice to involve all disciplines of risk management, pricing, marketing, underwriting, claims and experience monitoring.
- (b) The claims function, in the light of claims experience, should report on whether:
- The policy terms and conditions are clear in respect of the benefits provided and when they become due.
 - All exclusions to cover are clear and highlighted.
 - The period of cover is clear and precise.
 - Marketing and proposal forms are clear and precise, highlighting all relevant areas.

Model answer for question 2

Days of grace is a period of time beyond the premium due date, where no further premium has been received, when the insurer remains on risk. During this period the policyholder is allowed to pay the outstanding premium without cover lapsing and without the need for further underwriting.

Model answer for question 3

- (a) Epilepsy is a disorder where abnormal electrical activity in the brain results in fits associated with disturbance of consciousness, and abnormal movements/behaviour.
- (b) Grand mal epilepsy gives rise to major fits where the individual suddenly loses consciousness, with twitching of arms and legs for up to a minute.

Petit mal epilepsy is non convulsive, however an individual may become unaware of their surroundings and stare into space or 'freeze' – usually lasting less than 30 seconds.

Model answer for question 4

- (a) Domicile is the place in which a person has their fixed and permanent home and to which whenever they are absent, they have the intention of returning.

Residence is where a person lives.

- (b) S.19 of the Revenue Act 1889 provides that, where a person dies domiciled abroad, it will not be necessary to produce a UK grant to receive the money payable. This is in contrast to the general rule that an English grant must be produced to collect an English asset.

Model answer for question 5

The Binet staging system is used for the staging of chronic lymphocytic leukaemia (CLL).

The Binet staging system reflects the extent of the disease, stage A corresponds to low risk, stage B to intermediate risk and stage C is high risk.

The latest ABI definition of cancer states that Chronic lymphocytic leukemia is excluded unless histologically classified as having progressed to at least Binet State A.

Model answer for question 6

- The document provides model exclusions.
- Insurers are free to omit any of the model exclusions and may include additional exclusions other than those listed as model exclusions.
- All exclusions should be contained in one section in the policy and marketing printed material.

Model answer for question 7

- (a) Troponin is a protein that occurs exclusively in heart muscle. Its level will rise in the blood following death of, or damage to heart muscle. It is a particularly sensitive test and can detect smaller degrees of heart damage than evidenced by the older method of measuring enzymes such as creatinine Kinase.
- (b) The latest definition requires a certain level of troponin to be present before the claim is deemed valid. These are:
Troponin T > 1.0 ng/ml, or
AccuTnI > 0.5 ng/ml or equivalent threshold with other Troponin I methods.

Model answer for question 8

Pension payments should be treated in accordance with the relevant policy conditions.

Generally where a claimant is also in receipt of a regular pension payment in addition to their income protection payment, the claims assessor should only take into account any pension which became payable as a direct result of the claimant's incapacity.

Generally where a claimant was in receipt of a pension (whatever sort) prior to becoming incapacitated, then the amount being received should not be taken into account in either pre or post disability income assessments.

Thus the assessor is only concerned with pension payments that commence as a direct result of the incapacity giving rise to the claim.

Model answer for question 9

- (a) Parkinson's disease is a degenerative disease of the nervous system resulting in progressive disturbance of voluntary movement. It occurs as a result of a reduction in the production of a substance called dopamine which affects the transmission of messages within the brain.

The main symptoms of Parkinson's disease are as follows.

- Tremor - which usually begins in one hand or arm and is more likely to occur when the affected part of the body is at rest and decrease when it is being used. Stress can make the tremor more noticeable.
- Muscular rigidity or stiffness - people with Parkinson's disease often have problems with movement – they often have a hesitant shuffling gait, and have difficulties getting out of a chair, changing direction when walking etc. They may also have problems with making fine finger movements, facial expressions and can be prone to falls and accidents.
- Bradykinesia - movements can become difficult to initiate, take longer to perform and lack co-ordination.

Other symptoms can include tiredness, depression, and difficulties with handwriting, speech, and balance.

Parkinson's is a very individual condition and each person will have a different collection of symptoms and response to treatment. The rate at which the condition progresses, the nature and severity of symptoms also varies in each individual.

Dopamine precursor (levodopa) is the treatment of choice for patients disabled by Parkinson's disease. It tends to be more beneficial in the early stages.

There are no tests that can definitely prove that someone has Parkinson's disease so doctors usually base their diagnosis on medical history and a clinical examination of the person. Laboratory tests and scans may be carried out to rule out other potential diagnoses.

- (b) The limitation of benefit calculation at the outset of this claim in April 2008 should have been as follows.

Maximum benefit payable will be 50% of pre disability earnings less State Benefits received, other insurances, and any continuing income.

Pre disability earnings are evidenced in the accounts for the partnership for the year ending 1st March 2008, which show a profit of £60,000.

Policy conditions and prevailing philosophy will determine whether his income is deemed to be £60,000 or £30,000 (i.e. a 50/50 split with his wife).

Consideration needed to be given to whether this amount should be split between Mr and Mrs Jones as both are named as partners in the accounts.

The evidence provided is clear that Mrs Jones had no involvement in the business prior to Mr Jones's disability and therefore it could be argued that the full £60,000 should be considered to be his pre disability earnings. In order to do this the assessor would have to be satisfied that the spouse had no intention of actively participating in the business.

The limitation of benefit clause would therefore have been:

£60,000 x 50% = £30,000
Less State Benefits of £5,000
No deduction for other insurances or continuing income
Maximum allowable benefit £25,000

Policy benefit is £24,000 so full benefit is payable at the outset of the claim.

- (c) The limitation of benefit clause needs to be reviewed as we are now aware of continuing income arising from the business with effect from 1st April 2008.

It is clear that the new business arrangement has led to overall profits being maintained at their previous level despite the claimant's incapacity.

The assessor needs to consider what benefit should have been payable over the past 12 months based on these figures. They need to determine what share of the net profit should be attributed to the claimant – as he has clearly reduced his involvement in the business, and his wife has increased hers.

From the information given it would be fair to assume that there is now a 50/50 split of effort, and potentially profit should be shared in the same manner. This may be evidenced by a partnership agreement. This would leave a net profit of £30,000 attributable to the claimant to reflect his earnings since his return to work in a reduced capacity.

Applying the rehabilitation benefit clause the benefit payable subsequent to his return to work in April 08 should therefore be:

$$\frac{£60,000 - £30,000}{£60,000} \times £24,000$$

Resulting in reduced payment of £12,000 per annum.

If the claim has been paid in full since admittance the assessor will need to consider whether it is possible to reclaim the overpayment or offset it against future payments. The wording of the admittance letter, and whether payments were made 'on account' will need to be considered.

- (d) This claim needs to be managed carefully in the future – the claimant's medical condition is likely to deteriorate, but in the meantime he continues to work in a reduced capacity so the claim needs to be managed accordingly.

Periodic reviews should be undertaken to keep in touch with the claimant and his circumstances.

Medically reports should be requested periodically to determine the progression of the disease, and the effectiveness of the treatment. However it is unlikely that the claim would ever be considered medically invalid so medical reports providing a brief update on his condition is all that will be required. The claimant's own account of his symptoms will also be useful in determining how much work it is reasonable for him to undertake.

Financially the claimant should be asked to provide regular updates as to how his business is operating. Payments should be made on the basis of the rehabilitation formula – however these should be 'on account' so that any under or over payment can be rectified once later accounts are available and the financial performance can be determined. The claimant may have monthly management accounts that could be shared with the insurer to demonstrate the performance of the business.

It is likely that the claimant's capacity to work will reduce with time and therefore payments will increase to reflect this.

Model answer for question 10

Dear Policyholder

Thank you for your letter of dd/mm/yy date advising that you have recently been diagnosed with cancer and asking about whether it is possible to claim under the policies you hold with Good Life Insurance Company.

Our records indicate that you hold two policies with our company as set out below:

- Critical illness policy number 123456, providing life cover, critical illness cover and total disability benefit of £x. The sum insured is paid once, on the first of those events to occur – subject to fulfilment of the policy terms and conditions.
- Life protection policy number 987654, providing life cover and terminal illness cover of £y. Again, the sum insured is paid once, on the first of the above events to occur – subject to the fulfilment of the policy terms and conditions.

In order to decide whether or not you are eligible to make a claim under either policy it will be necessary for us to collect information from both you and your doctors. We will be seeking to establish whether the benefit definitions have been satisfied and whether your medical history is consistent with that declared at proposal. In the first instance, we need you to complete and return the enclosed claim form, which includes an authority form allowing us to approach your doctors for a report on your condition.

Further details about the benefits that can be claimed under each policy are set out below.

Critical illness policy number 123456

Critical illness cover extends to the range of conditions listed in your policy, this includes cancer. In order to have a valid critical illness claim for cancer, the policy definition of cancer must be satisfied. Cancer is defined in your policy in the following way:

Cancer – excluding less advanced cases

Any malignant tumour positively diagnosed with histological confirmation and characterised by the uncontrolled growth of malignant cells and invasion of tissue.

For the above definition, the following are not covered:

- All cancers which are histologically classified as any of the following:
 - non-invasive;
 - cancer in situ;
 - having either borderline malignancy; or
 - having low malignant potential.

- All tumours of the prostate unless histologically classified as having a Gleason score greater than 6 or having progressed to at least clinical TNM classification T2N0M0.
- Chronic lymphocytic leukaemia unless histologically classified as having progressed to at least Binet Stage A.
- Any skin cancer other than malignant melanoma that has been histologically classified as having caused invasion beyond the epidermis (outer layer of skin).

I should point out at this stage that the definition of cancer in your policy does include a number of exclusions, mainly relating to early stage cancers. We will be able to determine your eligibility to claim the critical illness benefit once in receipt of your claim form and the relevant medical reports from both your GP and your specialist.

Life Protection policy 987654

The sum insured under this policy of £y becomes payable in the event of your death or in the event of you being diagnosed with a terminal illness.

Terminal illness is defined in the policy as follows.

Advanced or rapidly progressing incurable illness where, in the opinions of an attending Consultant and our Chief Medical Officer, the life expectancy is no greater than 12 months.

You will no doubt appreciate that this is a very severe definition and the sum assured will only become due on death or you being diagnosed with a terminal illness that is one where survival is not expected to be longer than 12 months. We will be able to form a view on whether it is possible or not for you to make such a claim, when we receive the medical evidence to support your critical illness claim.

If your claim or claims are subsequently established to be valid under these policies, the sums of £x and/or £y will become payable and the policy and all the cover it provides will then cease.

Model answer for question 11

- (a) Typically a director's remuneration might comprise of salary and dividends.

They may also be entitled to benefits in kind e.g. company cars or private medical insurance, bonuses, pension contributions.

- (b) Company directors of a publicly quoted company are employees of the company that they work for.

Assessors will need to consider their IP terms and conditions and in house philosophy in order to establish how pre disability income is defined. Typically policy conditions will refer to 'gross income earned' although this term needs to be interpreted to reflect the remuneration structure of a company director. Some companies may have a definition for company directors which will make specific reference to salary and dividends.

The policy conditions will stipulate over what period pre disability earnings should be evidenced – this may be the 12 months prior to disability, or could be averaged over a longer period, say 2 or 3 years.

In establishing pre disability earnings the assessor should always limit their scope to *earned* income. The assessor would not normally take account of unearned income arising from their directorship in the form of dividends or share options.

Salary

A director's salary will be evidenced by their P60, and this amount may also be stated as a separate expense in the company's accounts. Payslips may also provide evidence of salary payments

Dividends

Many directors receive a proportion of their remuneration as dividends as these do not attract national insurance contributions. If these dividend payments are remuneration for work performed – as opposed to a return on a financial investment (i.e. unearned income) the assessor may look to take these in to account as earned income.

If dividends are to be taken as income then, in an ideal world, they must be paid out of the adjusted net profit of the business for that year. It is possible to declare dividends even if the business has made no profit. The money will come from the business bank account and could simply increase or create an overdraft. Dividends should therefore only be accepted as income if they are paid out of the net profit of the business generated in the pre incapacity period. Whether this is enforceable depends on individual policy wordings.

Dividend payments are evidenced on a document known as a P11D. Alternatively dividend vouchers may be supplied.

Benefits in Kind

Benefits in kind are non-cash benefits. These could be taken into account when assessing pre disability earnings if the assessor is satisfied that they cease to be received by the employee when IP benefit payments commenced.

For example, if an assessor allows an increase in benefit because an employee has a company car, then they would expect the car to be returned to the company in the event of incapacity that exceeds the deferred period in the policy. If this does not happen, then the employee would profit from receiving both the additional policy benefit and the car.

Benefits in kind may include:

- Company car (in some cases including fuel);
- Medical insurance;
- Home telephone expenses;
- Accommodation.

Provided these benefits were a continuous part of the employee's remuneration package, they can be taken into account in the financial calculation. They should only be included up to the value of the benefit used by the Inland Revenue to assess tax. One off payments such as relocation payments or benefits solely related to the duties of employment such as expense allowances, uniform allowances etc. should not be taken into account.

At the time of a claim, proof of these benefits can be found in form P11D issued by the employer.

Bonuses

Many employers pay bonuses and these monies could be taken into account provided there is sufficient evidence to show that these have been paid regularly. Care should be taken with any significant bonus only paid in the pre-incapacity period. This may represent manipulation of income to justify a higher benefit payment especially if the claimant has any degree of direct control over their remuneration package.

At the time of a claim the assessor will require proof of the bonus. The amount of a bonus should be included in the P60 issued at the end of the tax year but will not be shown as a separate item. It may be necessary to ask for the split between normal wages/salary and the bonus. This could be achieved by asking for the pay slip issued when the bonus was paid.

Pension Contributions

There are occasions when an employer has undertaken to make specific contributions to an employee's pension scheme. Where there is documentary evidence of this agreement that can be produced in the event of a claim, contributions may be taken into account as part of the applicant's earned income from the date the employer's contributions cease.

Model answer for question 12

The English and Scottish Law Commissions are in the process of consulting over the revision of insurance law in England. To date a number of issues papers have been published setting out initial thoughts on how the law may be revised. The purpose of the issues papers is to allow interested parties to comment on the initial thoughts of the Law Commission.

The current status is that a summary of responses was published by the commission in May 2008. A draft bill is to be prepared for the summer of 2009, when further comment will be invited from any interested party. The main features raised so far include the following.

The Law Commission has recommended abolishing the duty of disclosure, in so much as proposers should only be required to provide information in response to direct questions on the proposal form.

- There should be no duty on the consumer to disclose matters about which no question was asked on the proposal.
- Where the insurer asks a general question, the insurer should have no remedy in respect of an incomplete answer unless a reasonable consumer would understand that the question was asking about the particular information at issue.
- The law would still require a proposer who has stated a fact that at the time was true, but which ceases to be true before the proposal is accepted must still correct the statement. The insurer is expected to warn the consumer of this duty and the insurer is only entitled to a remedy where the proposer has acted unreasonably.
- The insurer will not have a remedy for misrepresentation unless the consumer made a misrepresentation which induced the insurer to enter the contract.

The current test for materiality is based on the prudent underwriter test and the Law Commission recommends that this should be replaced by the 'reasonable insured' test. This would ask what a reasonable insured in the circumstances would think was relevant to the insurer.

The proposer would have a duty to answer questions honestly and reasonably. An insurer would only have a remedy for non-disclosure or misrepresentation where it could show the following three points.

- The proposer made a misleading statement.
- It induced the insurer to enter into the contract and had the insurer known the full facts, it would have offered different terms.
- A reasonable person in the same circumstances would not have made the misrepresentation.

The Law Commission proposes 3 classifications of non-disclosure in place of the current regime adopted by the ABI and FOS.

- Innocent
- Negligent, and
- Deliberate or reckless

An insurer should not be able to rely on a misrepresentation if the insured was acting honestly and reasonably in the circumstances when they made the misrepresentation. Where the proposer has made a negligent misrepresentation, the court should apply a compensatory remedy by asking what the insurer would have done had it known the true facts. In particular:

- Where an insurer would have excluded a particular type of claim, the insurer should not be obliged to pay claims that would fall within the exclusion.
- Where an insurer would have declined the risk altogether, the policy may be avoided, the premiums returned and the claim refused.
- Where an insurer would have charged more, the claim should be reduced proportionately to the underpayment of the premium.

Insurers should have a right to avoid a policy where it has relied on a misrepresentation by the consumer proposer at the pre-contractual stage and the insurer shows that, on the balance of probabilities, the proposer made the misrepresentation:

- Knowing it to be untrue, or being reckless as to whether or not it was true; and
- knowing it to be relevant to the insurer, or being reckless as to whether or not it was relevant.

The insurer is entitled to a remedy for misrepresentation if two conditions are met.

- A reasonable insured in the circumstances would have appreciated that the fact which was stated inaccurately or was omitted from the answer would be one that the insurer would want to know about; or
- The proposer actually knew that the fact was one that the insurer would want to know about. The burden of showing that the proposer made a misrepresentation rests with the insurer.

In considering whether an insured acted with insufficient care in failing to give information, the judge or Ombudsman should consider how far it was reasonable for the insured to assume that the insurer would obtain that information for itself.

In particular, if the insurer indicated that it may obtain information from a third party (by for example asking the insured for consent to obtain it) it should not be allowed to rely on an honest misrepresentation if the insured reasonably thought that the insurer would obtain the relevant information from the third party before accepting the proposal.

The Law Commission has recommended the introduction of a 5 year cut off period in respect of life insurance. It is important to make a couple of points about this recommendation. It only relates to life insurance, not other forms such as critical illness and income protection. In addition, insurers would still be able to avoid policies for deliberate or reckless mistakes, but not for negligent ones.

Intermediaries should be regarded as acting for an insurer for the purposes of obtaining pre-contract information, unless it is clearly an independent intermediary acting on the insured's behalf. The test for determining whether an intermediary is independent and is acting as the consumer's agent should depend on whether the intermediary searches the market and conducts a fair analysis.

Model answer for question 13

The impact of a change of occupation and a change of residence will depend on the exact policy wording of the IP contract.

The policy may have certain geographical limits within which there is no impact in respect of the level of cover provided by his IP policy, nor any need to notify the insurer of his travel. The policy may or may not stipulate the need to tell the insurer of a change in occupation.

If there is nothing in the policy terms and conditions requiring notification of change in occupation and residence then his cover will remain valid subject to ongoing premium payments. However, the policyholder should be advised to:

- Consider the impact of his change of circumstances from a financial point of view may mean that this contract no longer suits his needs as an aid worker is likely to be paid significantly less than an accountant, and the limitation of benefit needs to be applied to this lower figure so he is aware of the value of any claim whilst he is in this role.
- What occupation his claim will be judged against in the event of a claim – the policy conditions may state that this is the job he was doing immediately prior to disability – or might state a specific occupation to be insured e.g. if he was an accountant when the policy was taken out this may be the insured role.
- Whether there are any restrictions on the location IP payments can be made to him, should a claim arise whilst he is in Romania.
- That financial proof of earnings will be required in the event of a claim and only a percentage of salary will be payable.
- If there are any relevant policy exclusions, specifically what the cover will provide in the event of him contracting HIV.
- If he chooses to cancel his policy now he must be aware that if he requires IP in the future he will need to be fully underwritten at that time – this will require HIV testing in view of his exposure in Romania– and will take in to account his health status at that time.

If there is a change of occupation clause within the IP terms and conditions the insurer may be able to adjust the terms of the policy to reflect the new risk. The details would have to be referred to underwriting to decide if the policy can continue, and if so, on what terms. The scenario proposed is unlikely to be acceptable and as a result the policy may cease.

Note that the ABI Statement of Best Practice for IP states that insurers must make it clear in the key features document that there is a change of occupation/duties clause, and how it defines occupation.

Most IP policies will restrict payment of benefit in relation to overseas residence or travel, typically to third world countries. The claims assessor will need to consider whether Romania falls within the scope of any geographical limits. Claim payments will sometimes be limited to a maximum of 13 or 26 weeks if the policyholder is outside of the agreed territories.

Model answer for question 14

- (a) It is not unusual when a life office makes an allegation of non-disclosure for the claimant (or their representative) to rebut the allegation by suggesting that they made a full and accurate disclosure to the sales agent, asserting that it was his failure to record the full details on the proposal form, rather than their failure to make an accurate disclosure. Any such allegation must be taken seriously and investigated fully with the sales agent. In the absence of evidence from the sales agent rebutting allegations it is likely that the FOS will presume what the claimant or his/her representatives state to be true.

In all cases where an allegation is made against the sales agent, be they a tied agent or IFA the agent should be contacted by the life office and provided with a copy of the allegations made. They should be asked specifically to provide a statement setting out their own position and response to the allegations made.

The consequences where the policyholder made a full and accurate disclosure but the sales agent failed to inform the insurer of the same facts are likely to be impacted on the legal status of the agent.

Generally speaking, IFA's are deemed to be the agents of the policyholder, although there may be circumstances where this is not held to be the case. Tied agents are though universally deemed to be the agents of the insurance company. The legal status of the intermediary will have consequences and to a large extent is likely to determine the remedies employed by the insurance company if the claimant's allegations are held to be the most credible.

In the majority of instances, an IFA will be deemed to have acted for the policyholder at proposal. Therefore any allegations about what information passed between the IFA and the insured will not be considered to have been passed on to the insurer. In such cases the claimant's remedy is likely to lie against the IFA not the insurer. In this case, therefore, if the insurer is satisfied that the agent was an IFA they are likely to (depending on the severity of the material non-disclosed) either make a proportionate payment or in the most severe cases, reject the claim and avoid the policy.

The claimant's representatives will then be required to take action against the IFA to recover any shortfall in the payment made by the insurer. Action to recover sums not paid, will usually be taken at the Financial Ombudsman Service, as IFA's are regulated by the FSA and therefore members of the FOS. Although the strict legal position is that failure by the IFA to pass on information to the insurer, should result in recovery by the claimant against the IFA, on occasion it has not been unheard of for the FOS to find that the insurer should make payment of the full sum insured. As indicated earlier the law of agency in such circumstances is complex and the FOS will look at all the circumstances surrounding the non-disclosure and make a determination as they think appropriate.

Where the alleged failure to pass on a disclosure relates to a tied agent the process is somewhat more straightforward for the insurer. A company appointed representative, will be deemed to have been the agent of the company for the purposes of effecting the policy. That means that disclosures made to the agent at the time of proposal will be deemed to be disclosures to the company.

Clearly any such details need to be followed up with the company agent and a full statement obtained from them addressing specifically the allegations made.

The tied agent can rebut the allegations made, either specifically or by stating that although the specifics of the case cannot be remembered, in accordance with procedures and/or training any such disclosures would have been recorded on the proposal.

Where such a rebuttal is not available, it will be presumed that the disclosure took place and that the insurer was aware of the facts in dispute. In such cases the insurer will have little option other than to accept that a full disclosure was made and then to pay the claim.

- (b) Other agency situations may arise when;
- The GP completes forms for the claimant, it may be argued that the GP is acting as the agent of the claimant in such situations
 - Medical examiners instructed by the insurer will be deemed to be agents of the insurer.

Failure of either the GP or a medical examiner to report back to the insurer details they were aware of as a result of a consultation or examination could have implications for the principal whether they be the insured or insurer.

If the GP fails to report facts, this may be deemed to be non-disclosure by the claimant. If the medical examiner fails to report back accurately to the insurer despite having been told the full details, the insurer will be unable to argue non-disclosure.